Document Control

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1. Background

The use of Epidural analgesia within General Surgery, Trauma and Orthopaedics has seen a large decline over the last few years. As a result, staff exposure to epidurals and competency levels in the management of epidurals has also declined. There are now many wards and departments across the organisation that have 'no' staff competent in epidural management which raises patient safety concerns.

Whilst epidural infusions have reduced across the organisation there are times where they play a vital role in enhancing patient's recovery where alternative analgesics are not appropriate or have failed to control pain adequately. It is, therefore essential to recognise their decline in clinical practice, lack of competent staff across the organisation, but also acknowledge their benefits to patient outcomes. The development of safe patient pathways is therefore essential to enable the continual use of epidurals, support educational programmes for staff and provide safe care in a safe environment.

Following a number of incidents and Serious Event Audits (SEA) it was agreed to restrict where epidurals were managed across the organisation and target training and competency across three key departments, Intensive Care (ICU/HDU), theatre recovery and Lundy Ward.

2. Purpose

The Standard Operating Procedure (SOP) has been written to:

- Ensure safe management and allocation of patients receiving epidural analgesia across the organisation.
- To identify referral process for patients being cared for outside of the specified departments who require insertion of an epidural for the management of pain.
- Identify training requirements and competency level of staff working within the specified departments managing and maintaining epidurals.

3. Scope

This Standard Operating Procedure (SOP) relates to the following staff groups who may be involved in the management of Epidurals in Theatre/Recovery and on Lundy Ward or Intensive care or involved in the transfer of care of a patient that requires epidural insertion:

- Anaesthetists
- Pain Management Nurses
- Registered nurses
- Theatre recovery Staff
- Medical Staff
- Surgical Staff

4. Location

Ideally all epidurals should be initially managed on ICU/HDU if bed space available. However, if ICU/HDU cannot accommodate the patient Lundy ward would be the next and only appropriate option available. Please refer to the contingency plan for increased demand on critical care services policy if ICU/HDU bed was preferable.

If a patient has an epidural inserted as part of their surgical procedure in theatre it is the responsibility of the siting anaesthetist to ensure they liaise with the necessary departments to ensure a smooth and safe transition of care and appropriate bed allocation, as per pain management guidelines p.31

Elective Orthopaedic patients requiring epidural analgesia will need to be cared for in an appropriate and safe environment where staff are competent in epidural management and assessment. Through agreement with the Orthopaedic Consultants this group of patients can ONLY be cared for on ICU/HDU and transferred back to ASU Orthopaedics once the epidural has been removed. In the event a bed is not available the patient will need to be cancelled for theatre, therefore consideration of all alternative analgesic options MUST be explored pre-operatively.

Epidurals can only be inserted in theatre or theatre recovery where insertion can be safely managed and appropriate/safe monitoring can take place.

Patients requiring epidural analgesia outside of Lundy ward/ICU/HDU environments will need to be referred to the anaesthetic/acute pain team for assessment of their pain, review of current analgesic regime and availability of appropriate and safe bed spaces to manage epidural analgesia. Liaising with the patients consultant and the clinical site management team is essential to ensuring a smooth and safe transition of care takes place.

5. Monitoring of Epidural Analgesia

As per the Royal College of Anaesthetics Core Standards for Pain Management services patients receiving epidural analgesia require daily reviews by either the acute pain management team or anaesthetic team. The acute pain team will review patients receiving epidural analgesia at least daily during their working hours (Monday – Friday 8am-4pm) and are contactable via bleep 279 during their working hours for troubleshooting.

The on call anaesthetic team are available outside of the acute pain team hours to troubleshoot and review patients as per recommendations in 4.1 for patients receiving epidural analgesia. The anaesthetic team can be contacted on bleep 504 for advice and support.

The acute pain team are responsible for ensuring that patients receiving epidural analgesia on Lundy ward are handed over to the on call anaesthetists at the end of each day for continuity of care.

Please refer to pain management guidelines on the monitoring and assessment of epidural analgesia

6. Education and Competency

Staff undertaking this task on Lundy Ward, ICU/HDU and Theatre Recovery must be able to demonstrate attendance at annual training provided by the acute pain management team or by the clinical educator on intensive care and continued competence as per the organisations policy on assessing and maintaining competence.

Staff working outside of these areas must demonstrate awareness of how to refer to the acute pain management team or anaesthetic team out of hours for advice and support when analgesic regimes have failed or are inadequately controlling pain, or where epidural analgesia may be deemed appropriate.

Staff across the organisation can access epidural awareness training via taught study days or through e-learning modules on STAR. Resource information can also be accessed on BOB through the Acute Pain Management Guidelines or the Acute Pain Management electronic resource file. Staff outside of Lundy ward, Intensive care and Theatre recovery will not be expected to directly care for patients with epidurals or hold competencies, but demonstrating awareness would be seen as beneficial.

It is the responsibility of the departmental leads for ICU/HDU, Lundy and theatre recovery to ensure new staff requiring training are flagged up to the acute pain team at the earliest opportunity. For these areas epidural training should be deemed as essential and be reviewed as part of their annual appraisal.

7. Audit

The Acute Pain Team will be responsible for auditing epidural usage annually as part of their annual report, investigating incidents with appropriate nursing and anaesthetic support and monitoring staff competency levels across the organisation.

8. References

- Royal College of Anaesthetists Core Standards for Pain Management Services

9. Associated Documentation

Northern Devon Healthcare NHS Trust Policies for:

- Acute Pain Management Guidelines
- Assessment and Maintenance of clinical competencies in Nurses, Midwives and support workers policy
- Medical Devices: training and assessment of competency policy
- Contingency plan for increased demand on critical care services policy

Appendix – Reference Guide

PATIENT PLACEMENT WITH EPIDURAL ANALGESIA

1.

- **PATIENT IDENTIFED AS REQUIRING EPIDURAL ANALGESIA** pre-op decision or ward based decision

2.

- **PRE-OP/THEATRE RECOVERY** - liaise with HDU/ICU for bed availability, if unable to accomodate patient liaise with Lundy ward for bed availability. This will need to be discussed with the clinical site management team (bleep 500). If no beds available on appropriate wards alternative analgesic techniques should be used. It's the role of the Anaesthetist to liasie with appropriate departments and staff before epidural inserted.
- **WARD BASED DECISION** - ***If patient on Lundy ward or ICU/HDU*** - Epidural insertion can go ahead in appropriate environment (theatre). Post insertion care can continue on ward. ***If patient is being cared for in another clinical environment/department*** the acute pain team/anaesthetic team will need to liaise with clinical site management team (bleep 500) for bed availability on Lundy ward or ICU/HDU. If bed not available alternative analgesics need to be considered.

3.

- **TRANSFER OF PATIENT FROM ICU/HDU** - ICU staff to liaise with clinical site management team and Lundy ward team for bed availability. The ward co-ordinator from Lundy ward should be notified prior to transfer to ensure appropriate placement of patients and allocation of staff.

PATIENTS RECEIVING EPIDURAL ANALGESIA NEED TO BE CARED FOR IN A SAFE ENVIRONMENT WHERE STAFF HAVE RECEIVED APPROPRIATE TRAINING AND ASSESSED AS COMPETENT IN EPIDURAL MANAGEMENT

LUNDY WARD AND INTENSIVE CARE NURSES HAVE RECEIVED SPECIALISED TRAINING AND ARE THEREFORE THE ONLY CLINICAL ENVIRONMENT EPIDURALS CAN BE PLACED WITHIN THE TRUST

***elective orthopaedic patients can only be cared for on ICU/HDU with epidural infusions**

EXCLUDING MATERNITY